COMPREHENSIVE HOSPITAL ADMISSION AND DISCHARGE SUMMARY

Synthetic patient: CASE-PAT-7408 | Fictional name: Jordan Example | Age: 58 years
Encounter: 2026-07-08 to 2026-07-12 | Facility: Fictional Cedar Valley Hospital
Service: General Medicine | Attending: Fictional clinician record | Record status: Final
This invented case is for software demonstration and testing only.

PRESENTING CONCERN AND HISTORY OF PRESENT ILLNESS

The patient presented with five days of increasing fatigue, reduced exercise tolerance, intermittent dry cough, and mild breathlessness when climbing stairs. The patient reported no chest pain, hemoptysis, syncope, unilateral leg swelling, or measured fever. Oral intake was lower than usual for two days. No recent travel or known sick contact was documented. A home temperature log was not available. Symptom chronology is based on the fictional history supplied at admission and was not independently verified in this example.

RELEVANT MEDICAL, SURGICAL, AND SOCIAL HISTORY

History listed in the record: hypertension, type 2 diabetes mellitus, and mild intermittent asthma. Prior appendectomy is listed in the surgical history. No prior venous thrombosis or heart-failure admission is documented in the supplied case. Former tobacco use is listed as approximately 8 pack-years; quit date is recorded as 2018. Alcohol history is not stated. Baseline exercise capacity and most recent outpatient renal results are not available here.

ALLERGIES AND HOME MEDICATIONS

Allergy list: trimethoprim-sulfamethoxazole - rash, reported by patient; reaction date unknown.
No medication allergy to penicillin is listed in this fictional chart extract.
Home medication list: metformin 500 mg by mouth twice daily; amlodipine 5 mg by mouth once daily; budesonide-formoterol inhaler, 1 puff twice daily; albuterol inhaler as needed. Adherence and last dose were not confirmed. No anticoagulant is listed on the supplied list.

ADMISSION ASSESSMENT AND INVESTIGATIONS - CONTINUED

INITIAL OBSERVATIONS

Blood pressure 146/88 mmHg; pulse 96/min; respiratory rate 20/min; temperature 37.2 C; oxygen saturation 95% on room air. Weight 82.4 kg. The record does not provide a prior baseline weight. These values are reproduced as recorded in the fictional case.

PHYSICAL EXAMINATION DOCUMENTED AT ADMISSION

Alert and conversant. No acute distress documented. Mild expiratory wheeze was heard bilaterally. No focal crackles were documented. Cardiac rhythm was regular in the note. No peripheral edema or calf asymmetry was recorded. Abdomen was soft without focal tenderness. Neurologic examination was described as grossly non-focal. Examination findings are the author's documentation and are not independently verified by this fixture.

SELECTED LABORATORY RESULTS FROM THE ATTACHED FICTIONAL REPORT

White blood cell count 10.9 10^9/L (4.0-11.0); hemoglobin 12.4 g/dL (12.0-16.0). Sodium 137 mmol/L (135-145); potassium 4.2 mmol/L (3.5-5.1); creatinine 1.08 mg/dL (0.60-1.10); glucose 184 mg/dL (70-99), reported high. Hemoglobin A1c 7.8% (4.0-5.6), reported high. C-reactive protein 18 mg/L (0-5), reported high. Troponin was below the laboratory reporting threshold on two samples. D-dimer was not ordered in this case. No additional reference interval is inferred where the fictional report does not print one.

OTHER TESTING

Electrocardiogram: sinus rhythm; no acute ischemic change described in the report. Chest radiograph: mild bibasilar linear opacity, favored to represent atelectatic change; no focal air-space consolidation or pleural effusion reported. Respiratory viral panel was negative for the targets tested. Blood culture showed no growth at 48 hours.

HOSPITAL COURSE, ASSESSMENT, AND MEDICATION RECONCILIATION

HOSPITAL COURSE

The patient was observed on the general medicine service. Inhaled bronchodilator treatment was given for wheeze, with improvement documented in the progress note. No supplemental oxygen was required. Intravenous fluids were given for reduced oral intake; oral intake improved. No fever was recorded during the documented stay. No antibiotic course was started because the treating team did not document a bacterial infection as established. The mild breathlessness improved but the record does not state that every symptom resolved.

DISCHARGE DIAGNOSES AND STATUS

1. Wheezing with mild intermittent asthma history - treated during the stay; the discharge record does not state that spirometry confirmed an acute asthma exacerbation.
2. Fatigue and exertional breathlessness - symptoms documented; a single definitive cause is not established in this record.
3. Type 2 diabetes mellitus with elevated glucose values in the supplied measurements.
4. Hypertension - history documented; no hypertensive emergency recorded.

No pulmonary embolism, bacterial pneumonia, acute coronary syndrome, or heart failure is documented as a confirmed discharge diagnosis in this fictional case.

MEDICATION RECONCILIATION - DISCHARGE INSTRUCTIONS

Continue metformin 500 mg by mouth twice daily with meals; the note requests routine outpatient review. Continue amlodipine 5 mg by mouth once daily. Continue budesonide-formoterol inhaler, 1 puff by inhalation twice daily. Albuterol inhaler, 2 puffs by inhalation every 4 hours as needed for wheezing. No new antibiotic or anticoagulant is listed at discharge. The medication list is reproduced from this synthetic summary.

DISCHARGE PLAN AND LIMITATIONS - CONTINUED

DISCHARGE CONDITION AND FOLLOW-UP

Condition at discharge is described as stable on room air, ambulatory, and tolerating oral intake. Primary-care follow-up was requested within 7 to 14 days; a booked appointment date is not present in this record. Repeat glucose review was requested by the treating team. Return precautions were documented as increasing breathlessness, chest pain, fever, fainting, or inability to maintain fluids. The record does not establish whether follow-up was scheduled or whether any requested action was completed after discharge.

DOCUMENTATION LIMITATIONS

No prior pulmonary-function report, echocardiogram, or ambulatory oxygen measurement is included. Smoking cessation status is based on the history provided. The cause of fatigue and exertional breathlessness remains incompletely characterized in the supplied note. This example preserves the source's uncertainty and does not add diagnoses or advice.

END OF FICTIONAL SUMMARY - TRAINING DATA ONLY